Situation Report: Ebola Virus Disease (Bundibugyo), DRC

PHEIC | WHO GRADE 3 | ACTIVE OUTBREAK | DAY 48 | 2026-07-02 05:00 UTC | Classification: Evidence Scan

⚠ **IMPORTATION ALERT — 24 June 2026:** France confirmed the first Ebola case outside Africa in this outbreak. A healthcare worker returning from a DRC humanitarian mission tested positive on 24 June. Viral load very low; stable; immediately isolated in a specialised facility. Contact tracing initiated (21-day monitoring). French MoH states no indication of local transmission. *Source: French Ministry of Health / WHO DON606.*

1,560	1354	401	16	36
TOTAL CASES	CONFIRMED	CONFIRMED DEATHS	CONTACTS TRACED	PCR MACHINES
confirmed + suspected	DRC 1333 + Uganda 20 + France 1	Death proportion: 29.6% (95% CI 27.2-32.1%, sampling only) | >=220 suspected deaths (DRC MoH)		

Executive Summary

An outbreak of Ebola virus disease caused by Bundibugyo ebolavirus (BDBV) continues to expand across 37 health zones in three provinces of the DRC (Ituri, Nord-Kivu, Sud-Kivu), with confirmed cross-border transmission to Uganda (20 confirmed + 1 probable, 2 deaths). **On 24 June 2026, France confirmed the first imported case outside Africa** in this outbreak — a healthcare worker returning from a humanitarian mission in DRC (very low viral load; stable; isolated; WHO DON606).

As of 2026-06-29, INSP reports 1,560 cumulative cases (1354 confirmed across 3 countries, 206 suspected) and 491 deaths. The confirmation rate remains low at 86.8%, reflecting limited BDBV-specific diagnostic capacity.

The highest attack rates are concentrated in Rwampara (874.5/100,000; n=536) and Mongbalu (379.5/100,000; n=604). Rwampara has no PCR machines deployed -- the most critical diagnostic gap.

No approved vaccine, therapeutic, or rapid diagnostic test exists for BDBV. WHO expert consultation (2026-05-28) prioritised MBP134, Maftivimab, and remdesivir for clinical trials; none are yet operational.

Sixteen outbreak genomes (May 2026) are available on Pathoplexus. Preliminary phylogenetic analysis indicates a distinct clade relative to 2007 and 2012 BDBV lineages, consistent with a separate introduction. Genomic coverage is approximately 1.2% and geographically biased.

Contact tracing is active (16 contacts traced cumulative) but follow-up completion rates are not reported. Conflict and displacement in Ituri and Nord-Kivu constrain response access.

Data currency: Epidemiological data from INSP SitRep via INRB-UMIE (2026-05-14 to 2026-06-29). Uganda figures from Uganda MOH via Reuters (2026-05-29). Genomic data from Pathoplexus (accessed 2026-05-30). All figures are provisional. This is an evidence scan, not a systematic review.

Epidemiological Situation

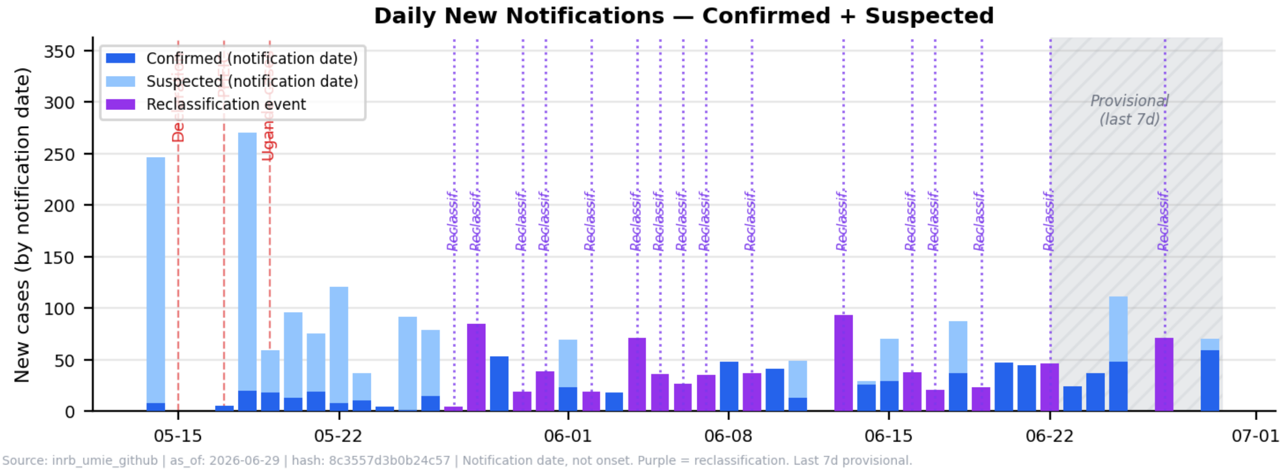
The outbreak was officially declared by the DRC Ministry of Health on 15 May 2026, with the index case -- a healthcare worker in Bunia, Ituri Province -- traced retrospectively to symptom onset on 24 April 2026. WHO declared a Public Health Emergency of International Concern (PHEIC) on 17 May 2026, two days after the national declaration.

Reported case accumulation remains rapid, although apparent changes between reporting dates are affected by reclassification and reporting lag. Between the first INSP SitRep (14 May) and the most recent available data (2026-06-29), cumulative confirmed cases increased from 0 to 1354, while suspected cases reached 206. The low confirmation rate (86.8%) reflects the limited availability of BDBV-specific RT-PCR, with only 36 PCR machines deployed across 19 health zones.

The confirmed-case death proportion is 401/1354 = 29.6% (Wilson 95% CI 27.2-32.1%). **This interval reflects binomial sampling uncertainty only** — for this near-complete count, the dominant uncertainties are right-censoring (pending outcomes), ascertainment bias (undetected deaths), and reporting delay; true uncertainty is substantially larger. This is a right-censored lower-bound indicator — not a case fatality ratio (individual outcomes not available; Ghani 2005; Nishiura 2009; WHO Ebola Response Team, NEJM 2014). **External reference only:** WHO estimates 30-50% fatality among confirmed; historical pooled BDBV 32.8% (95% CI 25.8-40.2%; PMC5124280). Confidence: low.

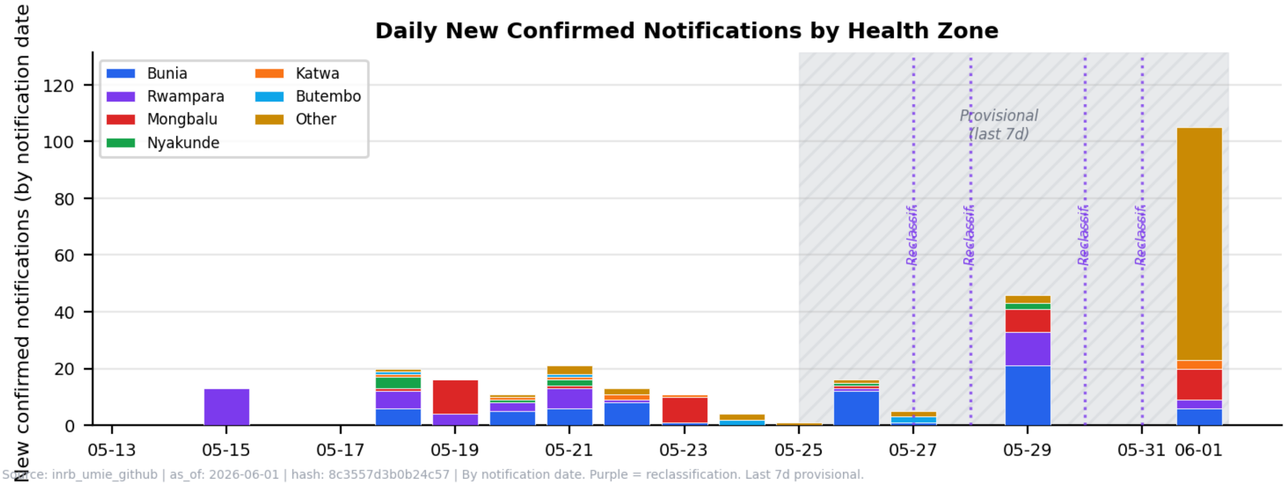
> **Death proportion summary:** 401/1354 confirmed cases have died (29.6%, Wilson 95% CI 27.2-32.1%). Right-censored lower bound — outcomes pending for active cases. Historical BDBV: 25% (2007 Uganda, 56 confirmed) to 50% (2012 DRC, 59 cases). McCabe et al. 2026 prior: 33% (95% CI 26–40%). Current outbreak is at the low end of historical range; interpret with caution. **Confidence: low.**

Epidemic Curve -- National (Confirmed + Suspected)



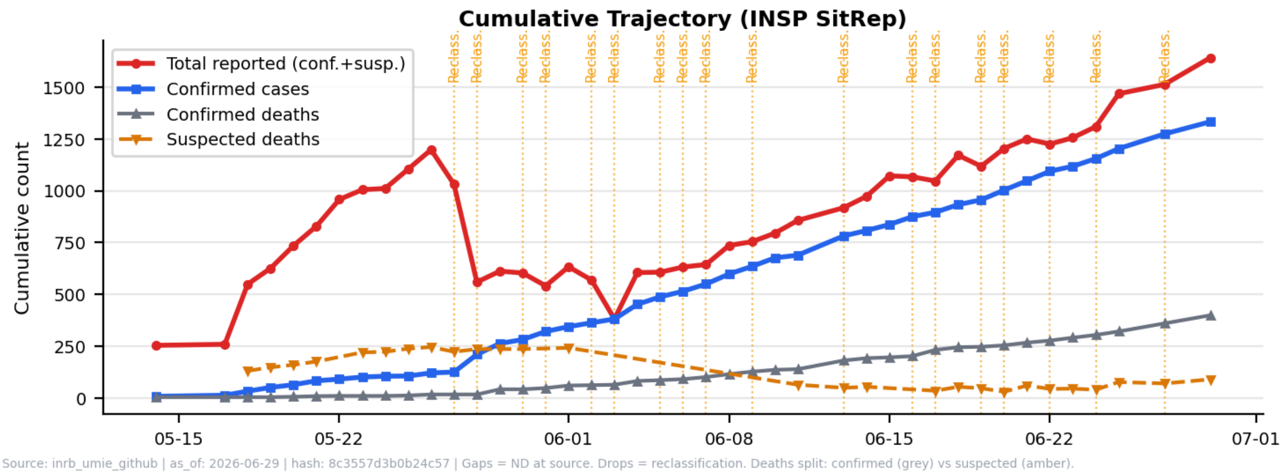
Source: INSP SitRep MVE 001-012 via INRB-UMIE GitHub. Notification date, not onset date. Dashed lines indicate key events.

Confirmed Cases by Health Zone



Top 6 zones stacked. Bunia, Rwampara, and Mongbalu account for 82% of confirmed cases. Source: INSP SitRep.

Cumulative Trajectory



Source: INSP SitRep via INRB-UMIE. Apparent decreases in cumulative totals between dates reflect reclassification of suspected cases.
Section sources: INSP SitRep via INRB-UMIE (2026-05-14 to 2026-06-29); CDC situation summary (07-02); WHO DON602, DON603, DON605 (2026-05-29), DON606 (2026-06-24 — France importation).

Geographic Distribution

The outbreak epicentre is Ituri Province, with the highest total case concentrations (confirmed + suspected, INRB-UMIE INSP 2026-06-29) in Bunia (626), Mongbalu (604), and Rwampara (536). When adjusted for population using WorldPop estimates, Rwampara has the highest attack rate at 874.5 per 100,000 -- more than 10.0x higher than Bunia (87.5/100,000) despite a smaller absolute case count, reflecting its small population denominator (61,295).

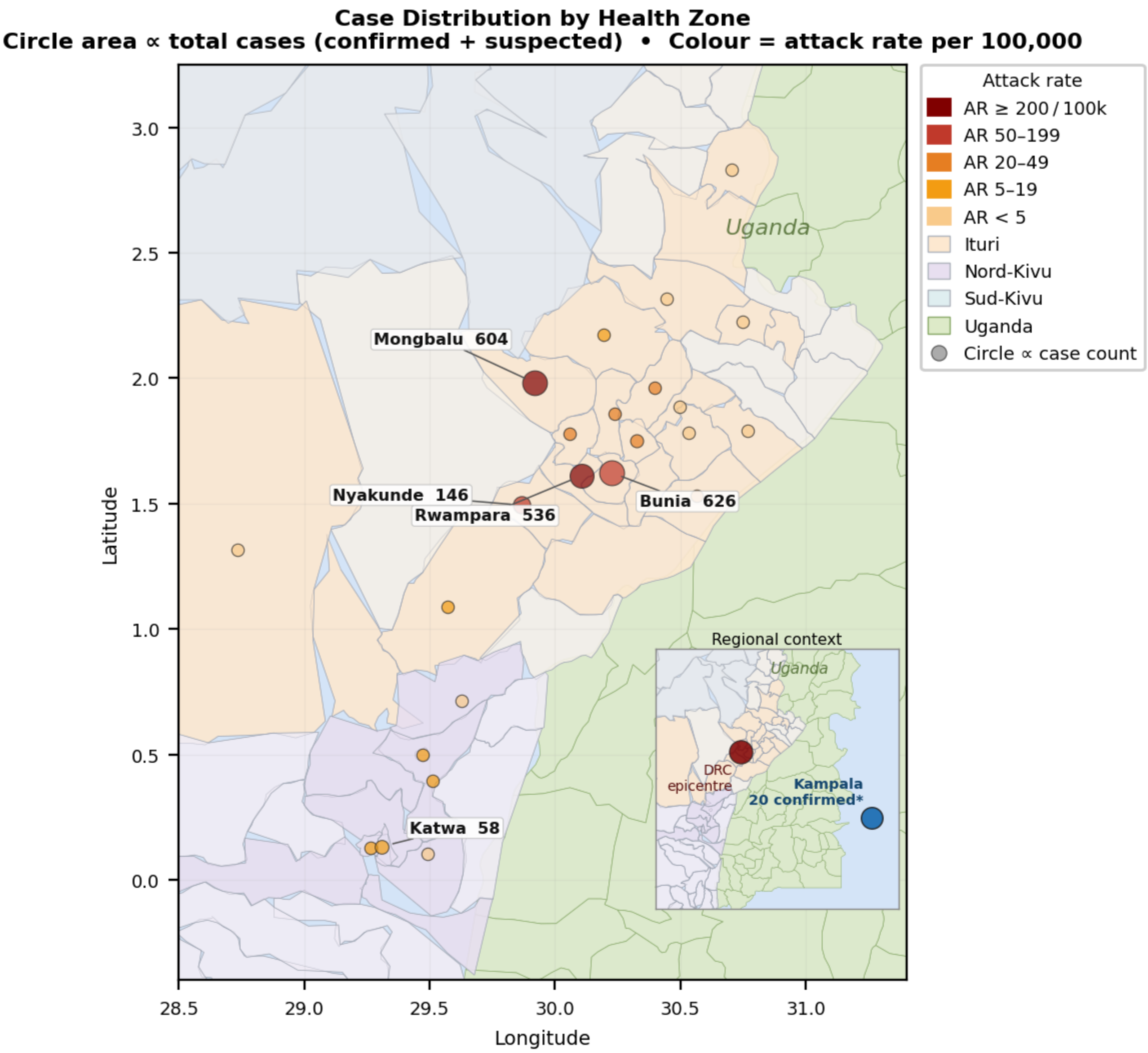
Nord-Kivu Province has confirmed cases in Butembo, Katwa, and Kalunguta. Goma-specific case status requires verification from DRC MOH/WHO subnational data; one case is recorded in the INSP SitRep but the city's epidemiological status should be treated with caution given its significance as a regional hub (~2 million population, international airport). Cross-border transmission to Uganda was confirmed on 19 May 2026, with 20 confirmed + 1 probable in Uganda (CDC 06-03; WHO DON605 05-29); 6 of the first 9 cases linked to DRC travel.

DRC Province Summary

PROVINCE	CONFIRMED	ZONES	STATUS
Ituri	997	22	Epicenter
Nord-Kivu	94	11	Expanding
Sud-Kivu	40	1	Limited

Source: DRC MoH / ECDC as of 2026-06-23. Ituri: epicentre (997 cases, 22 zones). Nord-Kivu: expanding (94 cases, 11 zones). Sud-Kivu: limited (3 cases, 1 zone).

Case Distribution Map



Health-zone centroids from INRB-UMIE shapefile. Circle size proportional to case count; colour indicates attack rate per 100,000. Population: WorldPop. Province-level allocations are working estimates. *Uganda: 20 confirmed national aggregate (CDC 07-02) — 9 in Kampala district, 11 unassigned; 2 death, 1 probable.

Health Zones by Attack Rate

HEALTH ZONE	CASES	POPULATION	AR /100K	PCR
Rwampara	536	61,295	**874.5**	0 (gap)
Mongbalu	604	159,147	**379.5**	2
Nyakunde	146	94,513	**154.5**	2
Bunia	626	715,169	87.5	10
Kilo	17	38,314	44.4	0 (gap)
Nizi	47	146,119	32.2	0 (gap)
Lita	29	91,650	31.6	0 (gap)
Mangala	24	82,616	29.1	1
Bambu	30	142,110	21.1	0 (gap)

Tchomia	13	64,496	20.2	1
Butembo	41	286,962	14.3	2
Nia-Nia	11	85,262	12.9	0 (gap)
Beni	22	231,984	9.5	2
Katwa	58	621,981	9.3	0 (gap)
Komanda	16	222,355	7.2	1

Attack rates use combined reported cases (confirmed + suspected) as numerator and WorldPop modelled population as denominator. Not adjusted for diagnostic confirmation or conflict-driven displacement. PCR machine count indicates deployment, not throughput capacity.

ETU Capacity Alert: Ituri ETU beds reached critical occupancy in mid-June 2026. MSF operates a 52-bed ETU at Mongbwalu General Referral Hospital and is establishing an additional 65-bed centre. The Bunia Elikya Hospital 36-bed ETU was at full capacity and is being expanded to 70 beds. WHO has installed individual isolation rooms and is scaling water/IPC infrastructure. Capacity expansion is underway but lagging case growth. *Source: MSF 2026, UN News 2026-06-17.*

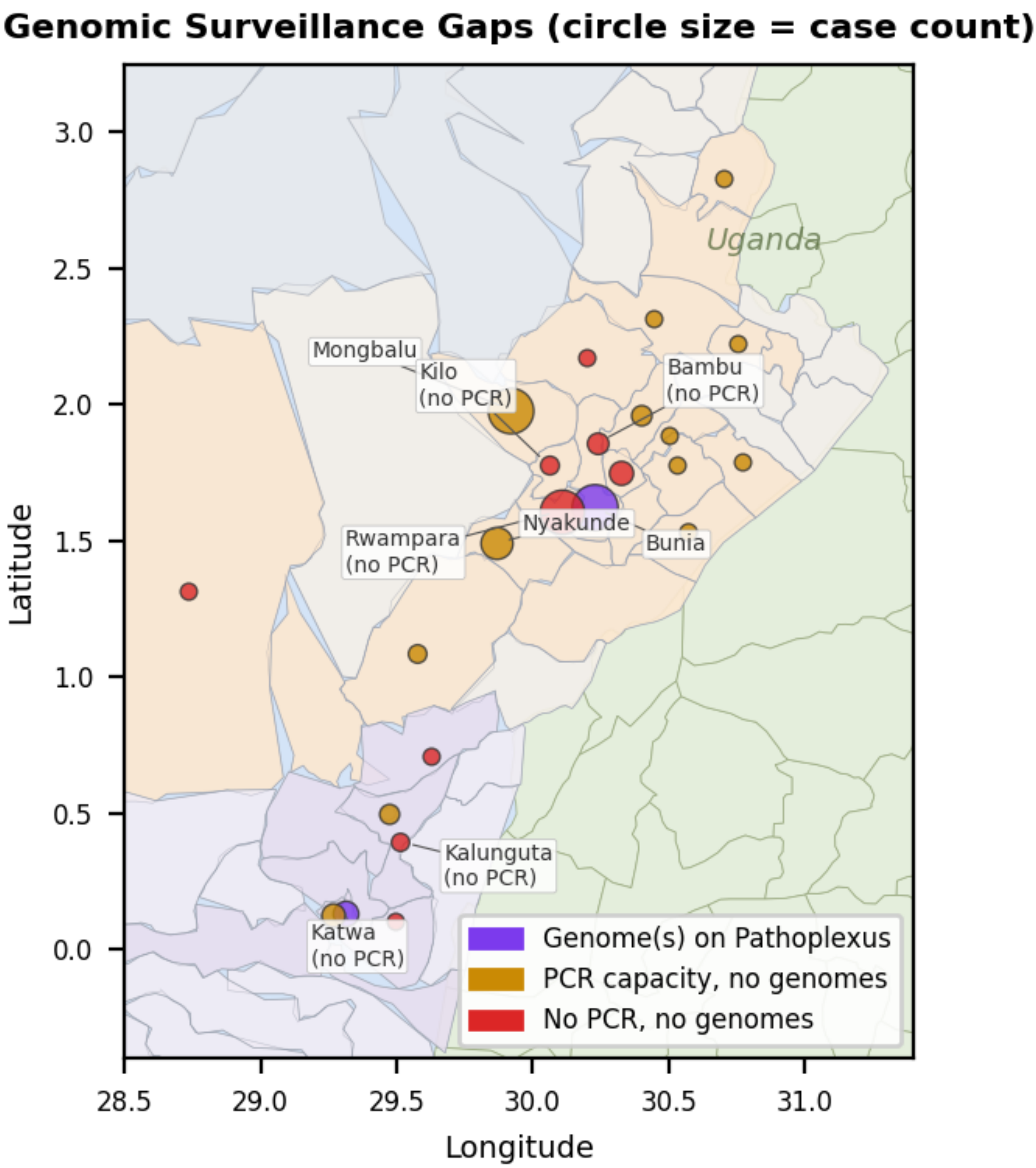
Section sources: INSP SitRep via INRB-UMIE (2026-05-14 to 2026-06-29); WorldPop population estimates (2020); WHO DON605 and CDC (07-02) for Uganda and province totals.

Genomic Surveillance

Sixteen BDBV genomes from the current outbreak (May 2026) are publicly available on Pathoplexus, released by INRB (DRC) and CPHL (Uganda) in three updates between 18 and 28 May 2026. Available genomes correspond to approximately 1.2% of confirmed cases (16/1354 as of 2026-06-29), though this should not be interpreted as representative sequencing coverage given geographic concentration in Ituri and temporal concentration in early May.

Preliminary phylogenetic analysis (Amuri-Aziza et al., Virological.org 2026; RACCOON pipeline: MAFFT + IQ-TREE2, HKY+gamma) indicates that 2026 genomes form a distinct clade relative to sampled 2007 Uganda and 2012 DRC historical lineages. The estimated tMRCA falls between late February and late April 2026, depending on evolutionary rate assumptions. This is consistent with a separate introduction; zoonotic origin is plausible for Ebola virus disease but cannot be directly demonstrated from human genome data alone. Evidence of host ADAR editing was observed in one genome (4 T-to-C mutations).

Sequencing Coverage Gaps



Purple: genome(s) available on Pathoplexus. Amber: PCR capacity but no genomes released. Red: no PCR, no genomes. Only 2 of 36 affected zones have genomes on Pathoplexus.

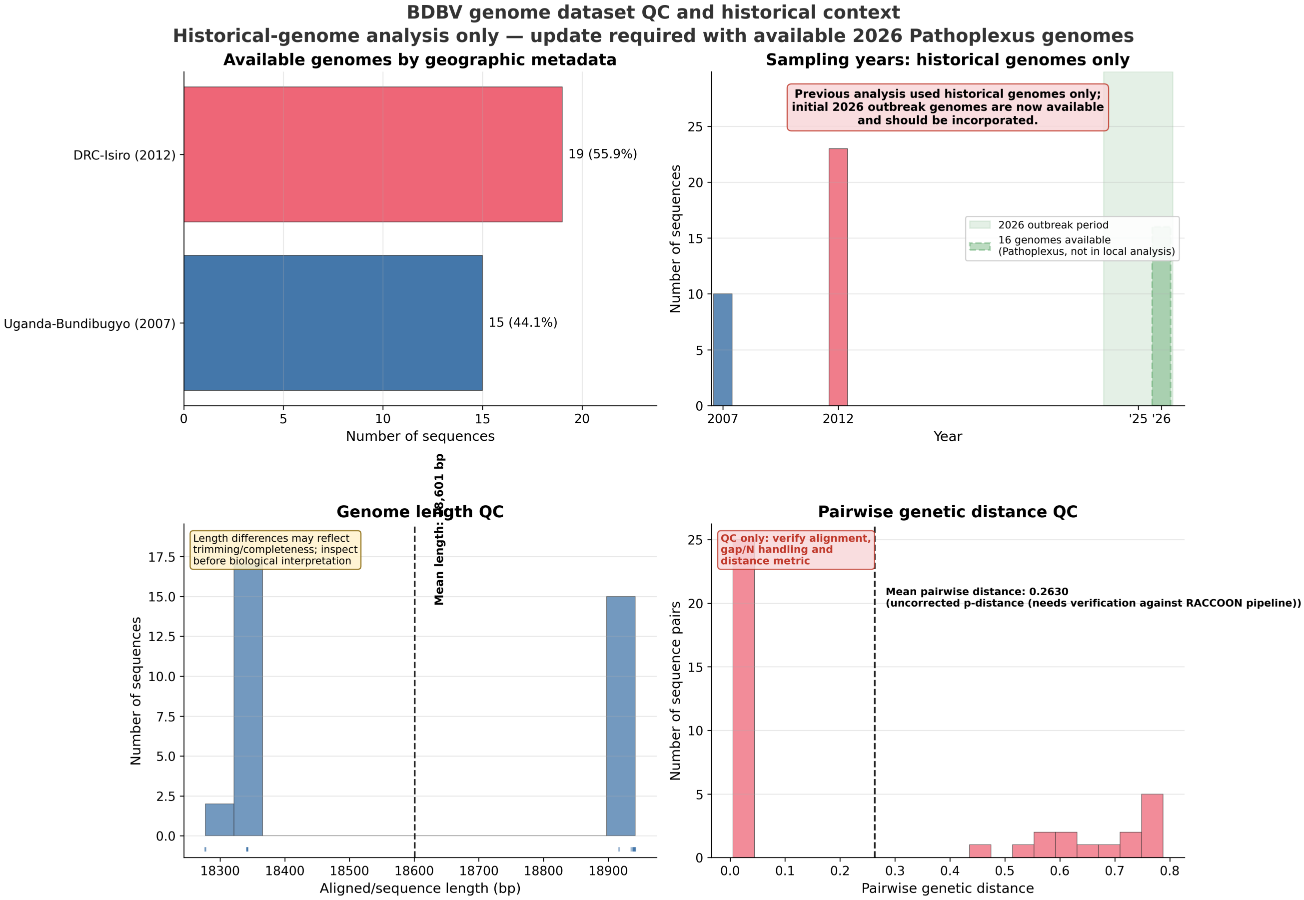
Selected Pathoplexus Accessions (2026 Outbreak)

ACCESSION	COUNTRY	LOCATION	DATE
PP_006XHL9.1	DRC	Bunia, Ituri	2026-05-03
PP_006XCJJ.1	Uganda	Kampala	2026-05-14
PP_006Y8NC.1	DRC	Katwa, Nord-Kivu	2026-05-06
PP_006Y8PA.1	DRC	Hoho, Ituri	2026-05-03
PP_006Y8Q8.1	DRC	Lumumba, Ituri	2026-05-03

+ 11 additional sequences (DRC Ituri, May 2026)

Data use: Pathoplexus "Restricted" licence. Contact Prof. Mbala-Kingebeni (INRB) / Dr. Ssewanyana (CPHL) before publication use. Sequence counts are snapshot-dependent.

Historical Genome QC Assessment



Local Atlas analysis: 34 NCBI GenBank sequences (2007 Uganda, 2012 DRC-Isiro). Pathoplexus 2026 sequences not yet incorporated into local pipeline. QC only -- not evidence for current transmission patterns.

Sequencing Turnaround

Median time from sample collection to Pathoplexus release is **15 days** (range 4-18, mean 13.6 days, n=16). The fastest turnaround was the Uganda sample (4 days, CPHL Kampala); DRC samples from INRB average 14-17 days. For genomic data to inform real-time response decisions, the target is under 7 days -- currently met only by Uganda.

Laboratory Capacity

36 PCR machines are deployed across 19 health zones, with Bunia accounting for 10 machines (28% of total capacity). Standard Ebola RDTs (SD Bioline, OraQuick) target Zaire ebolavirus GP and do not reliably detect BDBV. The testing cascade (samples collected, pending, rejected) is not published in INSP SitRep data; only positive (confirmed) counts are available. Estimated sample-to-result turnaround is 3-7 days including transport.

Critical diagnostic gap: Rwampara (536 total cases, highest attack rate 874.5/100k) has zero PCR machines deployed. 21 of 36 affected health zones lack any PCR capacity.

Section sources: Pathoplexus (accessed 2026-05-30); Amuri-Aziza et al., Virological.org (2026); INRB/CPHL sequence releases (2026-05-18 to 05-28); INSP SitRep via INRB-UMIE for PCR deployment data.

Countermeasure Status

Therapeutics

No approved BDBV therapeutic. WHO (2026-05-28) prioritised MBP134 (broad-spectrum mAb), Maftivimab (cross-reactive mAb), and remdesivir (antiviral) for clinical trials. Obeldesivir prioritised for post-exposure prophylaxis. Trials not yet operational.

Vaccines

No approved BDBV vaccine. Most advanced candidate: rVSVdeltaG/BDBV-GP (NHP survival benefit). Africa CDC estimates availability by end of 2026. Existing Ebola vaccines (Ervebo, Zabdeno/Mvabea) target Zaire ebolavirus and are not effective against BDBV.

Contact Tracing

DATE	CUM. TRACED	CUM. ISOLATED
2026-05-14	82	0
2026-05-15	0	4
2026-05-18	133	3
2026-05-19	0	1
2026-05-20	128	0
2026-05-21	0	0
2026-05-22	0	5
2026-05-23	183	1
2026-05-24	16	0
2026-05-25	16	--
2026-05-26	16	--
2026-05-27	16	--
2026-05-28	16	--
2026-05-29	16	--
2026-05-30	16	--

Caveat: Contact tracing figures are as reported in INSP SitRep aggregate tables. Cumulative counts are likely incomplete and non-comparable across SitReps — field teams may reset counts between reports, and sub-national follow-up data is not consolidated. The 16 figure represents what was reported in the last available SitRep entry, not a verified programme-level total. Follow-up completion rates: ND. Contacts-to-case ratio: ~0:1 (unreliable — see caveat).

Recommendations for Monitoring

1. Incorporate Pathoplexus 2026 genomes into local analysis for independent phylogenetic verification. **2.** Track WHO situation reports for updated confirmed case counts (current data: 2026-06-29, 3 days old). **3.** Monitor Uganda — Kampala is a regional air hub; further spread would broaden implications. **4.** Follow clinical trial operationalisation (MBP134/ Maftivimab/remdesivir). **5.** Verify Goma-specific case status from DRC MOH subnational data. **6.** Track diagnostic scale-up — the confirmation rate is the key capacity indicator. **7.** Assess conflict dynamics in Ituri/Nord-Kivu as the primary determinant of response effectiveness. **8.** France: monitor contact tracing completion over 21-day window; any additional imported cases should trigger updated risk assessment.

Data currency: Epidemiological data from INSP SitRep via INRB-UMIE (2026-05-14 to 2026-06-29; **data age: 3 days old**). CDC/DRC MoH figures (2026-06-24). France importation: French MoH / WHO DON606 (2026-06-24). Genomic data from Pathoplexus (accessed 2026-05-30). Uganda figures from Uganda MOH via Reuters (2026-05-29). All figures are provisional. This is an evidence scan, not a systematic review.

Risk Assessment and International Spread

The ECDC assesses the probability of infection for people living in the EU/EEA as **very low**, as BDBV transmission requires direct contact with the body fluids of a symptomatic patient. The France importation on 24 June 2026 — the first confirmed case outside Africa — confirms that the established pathway of healthcare worker exportation applies to this outbreak, consistent with 2014 West Africa precedent (US, Spain, UK, Italy: 4 imports; only the US reported secondary transmission, in 2 HCW).

Risk factors for further importations: Active HCW involvement (NGO staff, evacuation teams); incubation period up to 21 days allowing travel before symptom onset; low viral load at symptom onset may reduce but not eliminate detection sensitivity at borders. Voluntary and undeclared travel from affected areas carries higher undetected risk than official medical evacuations.

Risk factors limiting secondary transmission in high-resource settings: Effective contact tracing (Spain 2014: 87 contacts traced, 0 secondary cases); prompt identification and isolation; BDBV transmissibility below EBOV Zaire in community settings; no aerosol transmission documented.

France (24 June 2026): The imported case has a very low viral load, is in stable condition, and has been isolated. French health authorities state there is no indication of local transmission. WHO DG stated there is no need to panic. The key monitoring priority is contact follow-up completion over the 21-day incubation window. *Source: French MoH, WHO DON606, WHO DG statement 2026-06-24.*

Key Uncertainties

CATEGORY	STATUS	IMPACT
True outbreak scale	86.8% confirmed	Case count likely underestimated
Rt / growth rate	Not published	Cannot assess whether response is reducing transmission
Onset-based epi curve	Not available	Notification-date curve distorted by reporting lag
Demographics	Not available	Cannot target by age, sex, occupation
Genomic coverage	1.2% (geographically biased)	Insufficient for transmission-chain inference
Contact follow-up rates	Mostly ND	Cannot assess tracing effectiveness
Linelist	Not accessible	All analysis from aggregate reports
Population denominators	WorldPop (modelled)	Conflict displacement makes attack rates approximate
ETU capacity	Expanding — lagging case growth	Treatment access constrains survival outcomes
Further importations	1 confirmed (France); more possible	Requires 21-day contact monitoring per case

Disclaimer: This situation report is an automated evidence scan produced by the Atlas Outbreak Intelligence platform. It is not a systematic review and does not constitute official guidance from WHO, DRC MOH, Uganda MOH, MSF, or any other agency. Data quality depends on source reporting completeness and timeliness. INSP SitRep data is manually transcribed from PDF situation reports; transcription errors are possible. Population denominators are modelled estimates (WorldPop). Attack rates use combined case counts and are not adjusted for diagnostic confirmation or population displacement. Genomic and sequence counts are snapshot-dependent. All figures are provisional and subject to revision.

Sources: INSP SitRep via INRB-UMIE (github.com/INRB-UMIE/Ebola_DRC_2026), WHO DON602/603/605/606, ECDC Threat Assessment Brief, Pathoplexus, Virological.org, WorldPop, Reuters/WHO briefing, French MoH (2026-06-24).

Contact: office@loew-beer.at | Dashboard: outbreak.loew-beer.at | Open data: github.com/loew-beer/atlas-outbreak-data